

Hidden consequences of blunt trauma

CLINICAL M3-M5 | TYPE B | A focused decision guide

Different tissues different trajectories

Localise the damage

Clue	Site and consequence
Grey-white retinal change after impact	Comotio: photoreceptor outer-segment injury.
Profound loss with RAPD and a normal early disc	Possible traumatic optic neuropathy.
Wide ciliary-body band after healing	Angle recession: later glaucoma risk.
Phacodonesis or lens edge visible	Zonular injury; lens instability.

Follow the acute and delayed threats

Acute findings

Blood and pressure

Retinal whitening

RAPD / nerve injury

Delayed threats

Retinal breaks

Angle-recession glaucoma

Lens instability



Improved acuity is not the end of injury follow-up.

Original timeline schematic. Follow-up is tailored to the actual injury, not a fixed universal interval.

Comotio is not arterial occlusion

The trauma history, distribution and associated injuries matter. Macular involvement can limit recovery; assess for retinal breaks and other injuries rather than judging whitening alone.

Avoid an unproven restorative shortcut

Suspected traumatic optic neuropathy

- 1** Assess acuity, RAPD, colour and trauma imaging; exclude orbital compartment syndrome and a compressive cause.
- 2** Arrange urgent ophthalmology and trauma-team assessment. A normal early disc does not exclude optic-nerve injury.
- 3** Do not routinely give megadose corticosteroids for presumed visual benefit, especially with traumatic brain injury.

What to explain

Finding	Implication
No proven superior routine treatment	Observation and intervention decisions remain specialist-led.
Associated brain injury	Megadose steroids have a mortality harm signal.
Resolved anterior injury	Retinal and long-term angle follow-up may still be needed.

Angle recession

Extensive recession may precede glaucoma by years. A normal pressure reading after the acute event does not justify discharge from all future monitoring.